FOR IMMEDIATE RELEASE:

Title: Crisis in Access: Mental Health and Medical Care at Risk for PSEMAS Patients

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PSEMAS Patients at Risk as More Private Practitioners Withdraw from Unsustainable Contracts

Namibia's largest medical aid scheme, PSEMAS, is at risk of systemic collapse as private practitioners across the country withdraw from contracts due to unsustainable tariffs, regulatory paralysis, and a decade of rising costs without meaningful reform. The consequences will be devastating—particularly for rural communities and vulnerable groups dependent on mental health and general medical services.

Tariff Erosion and Unsustainable Practice Costs

- PSEMAS tariffs remain fixed at outdated 2014 NAMAF benchmarks—despite inflation and massive increases in operational expenses.
- Medical supply costs have more than doubled since 2014, while practitioner reimbursement has remained stagnant.
- Operating expenses have soared:
    - **Rent:** Average rent increased from N$3,200 (2014) to N$7,348 (2025).
    - **Fuel:** Petrol rose from N$11.91/litre (2014) to N$20.37/litre (2025).
    - **Food:** Staple prices, including eggs and meat, have doubled. Inflation remains high at 6.4% in 2025.
    - **Medical supplies & medicines:** The cost of consumables and pharmaceuticals required to treat patients has more than tripled since 2014. Items like sutures, gloves, syringes, and basic medications now cost 200–300% more—yet practitioners are still reimbursed at 2014 levels.

- **Staff salaries:** To retain qualified nurses, receptionists, and support personnel, practices have had to increase wages in line with national cost-of-living pressures. Salary increases of 60–100% since 2014 are now standard in the sector.
- **Combined burden:** The cumulative rise in these essential inputs—rent, fuel, food, salaries, and medical supplies—has rendered service delivery at 2014 tariffs economically impossible.

Systemic Failures and Regulatory Gaps

- PSEMAS contracts prohibit tariff adjustments, locking practitioners into uneconomical rates.
- The Namibia Competition Commission (NaCC) has flagged fixed-price contracts as potentially unlawful under the Competition Act—yet no action has followed.
- Practitioners who exit the PSEMAS network cannot claim for patients, shifting the entire burden onto public servants, who must then pay the full, market-related rate—rather than a 5% co-payment on an outdated tariff.

Administrative Burden and Contracting Vows

While private providers strive to remain part of PSEMAS to ensure access for low-income patients, the operational barriers are both cumbersome and outdated:

- **Manual claims submission**: PSEMAS still requires physical submission of all claims. Nothing is paperless. This outdated system contributes to frequent claim losses, rejections, and resubmission burdens. Practice staff must make multiple copies, track hand deliveries, and adjust internal processes simply to accommodate this inefficiency.
- **Increased admin load**: Practice managers and medical secretaries spend significantly more administrative time on PSEMAS claims than on any other fund—adding costs with no compensation.
- **Contract renewal process**: The process of remaining a contracted provider is unnecessarily complicated, bureaucratic, and demeaning.
- **Rigid and impractical co-payment structure**: Providers are contractually required to charge a 5% co-payment. However, many patients default on these small payments, and practitioners are forbidden from waiving or adjusting them. The result is increasing volumes of bad debt, for which providers have no remedy.

Declining Practitioner Participation

- PSEMAS-contracted providers dropped from 93% in 2020 to 75.6% by November 2024.
- Of those remaining, 16% have confirmed they will not renew in 2025, citing stagnant tariffs and unsustainable cost pressures.
- This means less than 60% of providers are expected to remain by the end of 2025.
- Without reform, complete collapse is projected by 2027, especially in rural areas, where providers are already stretched thin and funds for gap cover options are low.
- This crisis is not caused by high medical costs, but by inefficient FWA control, unregulated, and sometimes corrupt administrative structures.

Market Saturation and Mismanagement

- The **HPCNA** has aggressively expanded registrations as part of its strategic targets, overwhelming the private sector without assessing absorption capacity.
- **NAMAF** continues to issue practice numbers without ensuring a sustainable funding or service model.
- Most new practitioners end up serving PSEMAS patients, worsening the overutilisation of shrinking financial resources.

Fraud, Waste, and Abuse (FWA)

While other countries such as Botswana and South Africa enforce strong governance in public servant health schemes. South Africa's public servant medical aid (GEMS) pays market-related rates, maintains a 31% reserve ratio—well above the 25% industry norm—and is empowered to protect both its members and providers through transparent, accountable systems. Namibia continues to fall behind:

- Over N$7 billion has been spent on PSEMAS since 2016 (IMF).
- Yet, between 2020 and 2022, N$1 billion was paid to ghost beneficiaries (Deloitte).
- An estimated N$900 million is lost annually due to fraud and mismanagement (IMF, 2018).
- 32,000 non-contributing individuals are receiving benefits (MOF, 2024).
- Investigations to date have identified 84 practitioners linked to fraudulent activity under the PSEMAS scheme rooted in weak benefit controls and an outdated oversight framework.

- Whistleblower accounts and audits confirm that administrators, practitioners and members are implicated in fraud —a pattern already highlighted by former Finance Minister Schlettwein: *"Everyone had a hand in the till."*

Despite two decades of workshops and reports, no modern, efficient fraud prevention systems have been implemented.

Impending Collapse and Public Health Crisis

- If systemic issues are not urgently addressed, PSEMAS faces collapse by **2027**.
- Members who can afford gap-cover insurance may retain access to essential services—but mental health, chronic care, and rural services will no longer be viable.
- Already, suicide, domestic violence, and sexual assault are on the rise—symptoms of an increasingly fragile mental health support system.
- The burden is shifting to an already overwhelmed public health sector, which is underfunded and often relies on volunteer private practitioners to fill critical staffing gaps.
- About 300,000 public servants and their families risk losing access to present level of private care without realising the scheme is on the brink of failure.

Call to Action: Establish a Medical Control Board

The Namibia Private Practitioners Forum (NPPF) has, since 2013, consistently called for the establishment of a statutory Medical Control Board—a governance structure in place in countries like Botswana and South Africa regulating tariffs, coding structures, monitor regulatory compliance, market sustainability - a role neither NAMAF nor NAMFISA is mandated to perform creating a statutory vacuum.

In August 2024, NAMFISA confirmed the importance of establishing a Medical Control Board. The Financial Institutions and Markets (FIM) Act, passed in 2021, already provides the necessary legal framework to regulate fund administrators and impose significant penalties for fraud and abuse. Yet four years after its approval, the FIM Act remains unenacted, and the healthcare sector continues to operate in a legal and technological vacuum.

Going forward, paper-based claims must be replaced with secure blockchain ledger systems and AI-driven fraud detection, enabling real-time oversight and reducing the vulnerabilities that allow present systemic abuse and administrative inefficiencies to persist.

Conclusion

If meaningful regulatory reforms are not enacted — including the long-overdue establishment of a Medical Control Board, enforcement of administrator oversight under the FIM Act, and eradication of systemic data anomalies such as ghost recipients — then efforts to restore service delivery at sustainable tariff levels will remain futile. These interventions are not merely desirable; they are indispensable. Without them, even core programs like PSEMAS risk structural collapse — a scenario the Government still has the power to prevent—if it chooses to act with resolve and clarity.

Contact:

Dr Jürgen Hoffmann
CEO – NPPF
Cell: 081 1242884
Email: nppfmanagement@gmail.com